

NTLA pitch deck

PITCH — NTLA — VERDICT

PROPOSED — PENDING ADJUDICATION — Not a dislocation; a fairly-priced binary at the top of its own supply lid. Starter-sized, but only on a pullback into \$10.75–\$11.75; no purchase at today's \$12.84.

What we believe

Intellia is a pre-revenue gene-editing company with one drug that just won a large Phase 3 and one drug that could be much bigger but is only three-quarters owned.

The near-term drug, lonvoguran ziclumeran (“lonvo-z”), is a one-time treatment for hereditary angioedema — a disease where patients get sudden, sometimes dangerous swelling attacks. The Phase 3 (HAELO) reported on 2026-04-27: 62% of treated patients were both attack-free *and* off all other therapy, versus 11% on placebo. That is a strong result. A filing with the FDA is being submitted on a rolling basis, and the company has guided to FDA acceptance of that filing in the second half of 2026, with a self-run commercial launch in the first half of 2027.

The bigger drug, nexiguran ziclumeran (“nex-z”), targets ATTR cardiomyopathy — a heart disease where the existing branded drug alone does over \$5 billion a year. Intellia keeps 75% of it; Regeneron funds 25% of development and takes 25% of profits.

What we believe, specifically:

- 1. The company's own actions set the price.** Two days after the Phase 3 win, Intellia sold 19,255,815 shares at \$10.75, raising about \$194.6 million net. The stock fell 12% on the day of the good data. Separately, it sold 2,612,344 shares through an at-the-market program at an average of \$13.21, and it still has \$368.5 million of that program open. So the last two prices at which real size actually changed hands were \$10.75 and \$13.21. Today's \$12.84 sits between them. That is not a stock that has been left behind; that is a stock trading inside the range its own issuer has been selling into.
- 2. The cash position looks better than the business is.** Cash went up quarter-over-quarter, to \$628.4 million. But the company burned \$200.5 million of operating cash in the first half of 2026. Cash only rose because the stock sale and the at-the-market sales landed inside the period. Strip out financing and cash fell roughly \$170 million in six months.
- 3. The runway is roughly six quarters — about Q1 2028 — and it is not stable.** Intellia is expanding its ATTR Phase 3 (MAGNITUDE) from 765 patients to about 1,200. That is a 57% larger trial in a program with more than 650 patients already enrolled. That expansion costs money and pushes the readout later. It is the single clearest reason the remaining \$368.5 million of stock-selling capacity will get used.

4. **On honest math, the stock is worth roughly what it costs.** Value the HAE drug off the Street's own bearish peak-sales estimate (\$500 million a year, Evercore ISI), add net cash of \$4.48 a share, and you get about \$13.42 — five percent above the tape. Push the sales multiple up or credit the ATTR program at anything above zero and you get more. That range of answers is the honest answer: fair, with real optionality, not cheap.
 5. **Therefore: this is ownable, small, on a pullback.** We would own it because a 62%-vs-11% Phase 3 with a filing already going in is a genuine asset and the ATTR program is free at today's price. We would not pay up for it, because the issuer is a mechanical seller above \$13.21 and the runway forces more selling.
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What the market believes

The market is split, loudly, and both sides are fully staffed.

- **Sell-side:** 19 analysts publish an average target of \$23.94; the median of the covering set is \$17. The full range is \$8 to \$95. A range that wide is not a view; it is thirty people modeling different companies.
 - **Bulls** model the ATTR opportunity as a multi-billion-dollar franchise and treat lonvo-z as the funded on-ramp. The most aggressive published work carries multi-billion revenue by 2029.
 - **Bears** point at adoption, not data: there are roughly 12 approved HAE therapies as of 2026, three of them approved in 2025 alone (Andembry in June, Ekterly in July, Dawnzera in August). About 70% of US HAE patients say they prefer an oral preventive pill. Selling a permanent, one-time, irreversible liver edit into a non-fatal, well-managed disease where patients like their pills is a hard commercial sell, regardless of trial results.
 - **Short sellers** are at roughly 38% of the float — an enormous position that says a large, well-resourced group expects either a financing, a commercial disappointment, or a safety event.
 - **The most honest read of the price** is that the market has already decided this is a coin flip on commercial adoption, and is paying about the value of the HAE drug alone at conservative penetration, with the heart program thrown in free because nobody wants to pay for a trial that just got 57% bigger.
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Why we might have edge

Being candid: our edge here is small, and it is not an information edge. It is three things.

1. **We anchored to the right price.** Most write-ups on this name anchor to the \$17.84 high set on 2026-06-21, when the stock jumped 29% on additional Phase 3 data plus the rolling filing news, and then describe the drop to \$12.84 as a “fade on no new news.” That is wrong, and it is the kind of wrong that produces bad entries. The float never absorbed \$17.84 — that was a one-day news spike. The prices at which size actually cleared are \$10.75 (a marketed offering, book

open, post-data) and \$13.21 (the average of the company's own drip-selling). Anchoring to those instead of to the high changes the entry band by roughly 30%.

2. **We found the burn shock before it hits the run rate.** The MAGNITUDE expansion from 765 to ~1,200 patients is disclosed but does not appear in any trailing cash-burn number, because it hasn't been spent yet. Anyone computing runway from the last four quarters of cash flow — which is the standard method, and which reported burn *falling* 19% year-over-year — gets an answer that is structurally too optimistic. The direction of travel on spend is up, not down, and the disclosure is public.
3. **We are willing to say “fair” and act small.** The two disciplined answers here are “own a little, cheap” and “own nothing.” Most of the sell-side range implies neither. The wide dispersion is itself the tell that nobody has a defensible number.

What is **not** edge: we do not have a proprietary view on HAE penetration, on ATTR readout timing, on payer pricing, or on the safety mechanism. Our fair value sits inside the published Street range. If the thesis works, it will be because the entry price was disciplined, not because we knew something.

Why it might be priced in

Stated plainly, because this is the stronger side of the argument at \$12.84:

- **The issuer is the marginal seller.** \$368.5 million of stock-selling capacity remains open, and the company has already demonstrated it will use it at \$13.21 and price a full offering at \$10.75 within 48 hours of good news. That is a ceiling with a mechanism, not a sentiment ceiling. Any rally toward \$13–14 invites supply.
- **The good news is out.** The Phase 3 read out in April. The additional data and the rolling filing landed in June. The stock fell 12% on the Phase 3 win itself. When a market sells positive pivotal data, it is telling you the data was expected and the commercial question is what's unresolved.
- **Our valuation agrees with the Street's median.** Median target \$17, our central estimate \$13–19. There is no gap for us to be right about.
- **The competitive picture is public and bad.** Twelve therapies, three 2025 approvals, entrenched incumbents (Takhzyro around \$1.7 billion, Orladeyo around \$437 million and growing 51%), and a documented patient preference for oral dosing. None of this is hidden.
- **Zero commercial history.** Intellia has never shipped a product — no approvals, no recalls, no enforcement history, because there is nothing to have a history about. The company is building a launch organization from scratch for early 2027 (general and administrative spend is up 39% year-over-year). First launches by first-time commercial organizations routinely disappoint, and nothing in the current price appears to discount that.
- **The safety issue is managed, not eliminated.** There was a patient death and a clinical hold in the ATTR program. The FDA-aligned mitigation package includes excluding patients with certain liver abnormalities and short-term prophylactic steroids. That is a real prospective protocol change, which is better than we initially credited — but it is a liver-abnormality

exclusion, not a genetic screen that identifies who is at risk. The underlying susceptibility is still not understood.

- **38% short interest cuts both ways.** It can fuel a violent rally on a filing-acceptance announcement. It also means a very large, well-informed group has done this work and reached the opposite conclusion.

What we checked ourselves

Including where our own analysts got it wrong. Both benches worked from the same evidence pack; both corrected it; both also made errors that the other caught.

Claim	How checked	Found	Confirmed?
Share count	10-Q cover page, SEC 10-Q filed 2026-08-06	140,126,693 shares at 7/31/26	Yes. Our evidence pack's share series stopped at 103.7M (mid-2025) — 26% stale. Both benches caught it. Any valuation built on the pack alone would have been 35% too high per share.
Operating cash burn	10-Q cash flow statement	\$200.459M used in H1-2026 (~\$100.2M/quarter)	Yes.
"Burn is only \$91M/quarter" (original thesis brief)	Compared to the filed cash flow statement	Wrong, and wrong twice. The \$91M figure was built by subtracting stock compensation from operating cash flow — but operating cash flow already adds stock comp back. Double-counting. The real number is \$100.2M/quarter. The error flattered runway by about a quarter.	Refuted — this was our own brief's error, caught by both benches
"Cash went up, so the business is funded"	Cash flow statement, financing section	Cash rose only because \$194.6M of stock sales and \$33.6M of at-the-market sales landed in the period. Ex-financing, cash fell ~\$170M in six months.	Yes
Post-Phase-3 offering size and price	10-Q equity note + company pricing release, 2026-04-29	19,255,815 shares at \$10.75 (16,744,187 base plus a fully exercised 2,511,628 option), ~\$194.6M net, priced two days after topline	Yes — and our red bench understated it. Red reported 16.74M shares, missing the exercised over-allotment. The correction makes red's own argument stronger, not weaker: 15% more supply cleared at \$10.75 than red claimed.
At-the-market program status	10-Q equity note	2,612,344 shares sold at \$13.21 average in 2026; \$368.5M capacity	Yes. This is the single most decision-

Claim	How checked	Found remaining	Confirmed? relevant fact in the file.
“Runway at least into 2028”	Company release 8/6 vs. the 10-Q	The language matches across both venues — no inconsistency in the claim itself. $\$628.4\text{M} \div \$100.2\text{M}/\text{quarter} = 6.27 \text{ quarters} \rightarrow \text{about Q1 2028}$.	Yes, with about one quarter of margin
“Burn is re-inflating” (red bench)	Year-over-year comparison in the same 10-Q	Wrong. H1-2026 burn was \$200.5M against H1-2025 of \$248.6M — burn is down 19% . Red asserted the opposite without pulling the comparative column.	Refuted — our own bench’s error. The runway arithmetic survives; the trajectory claim did not.
Ownership of the ATTR program	10-Q collaboration note	Regeneron takes ~25% of worldwide development costs and commercial profits. Intellia owns 75%.	Yes. Also explains part of why burn fell.
Our own valuation table	Audited the inputs	Arithmetic correct, but it valued the HAE drug at $2.5 \times$ peak annual sales — a number with no source — and assigned zero to a 75%-owned Phase 3 heart program. At $3.5 \times$ the same \$500M peak gives \$16.98/share; at $4 \times$, \$18.80. The headline conclusion (“+50% needs \$830M peak sales”) is an artifact of an unsourced multiplier.	Partly refuted — our own bench’s error. Red conceded the ATTR program was a free option and then priced it at zero anyway.
Enterprise value	Recomputed from verified share count	$140.13\text{M} \times \$12.84 - \$628.4\text{M} = \mathbf{\$1.17B}$ (the original brief said \$1.09B)	Yes, corrected
“We just agree with the Street”	Compared our number to published targets	Our \$13.42 central estimate is 44% below the Street mean of \$23.94 and 21% below the \$17 median , and we have a named reason (a decay-style sales multiple plus zero ATTR credit). That is a divergence, not agreement. Red mislabeled its own most original work as consensus.	Refuted — our own bench’s error. Being inside an \$8-to-\$95 range means nothing; that range is too wide to be informative.
ATTR Phase 3 size	Registry and company release on the 2026-03-02 clinical hold lift	MAGNITUDE being amended to ~1,200 patients from 765, with >650 already enrolled, subject to regulator review	Yes — and both benches initially missed it. Found only on the blue bench’s second pass. This is the real cost and timing shock.
“Safety is monitoring, not exclusion”	Same company release	Both benches were wrong. The FDA-aligned package expressly includes excluding patients with certain liver abnormalities plus prophylactic short-term steroids — a genuine prospective protocol exclusion. It is not, however, a genetic screen; the retrospective HLA correlation remains a correlation.	Partly refuted — shared bench error. Concern reduced, not removed.
HAE competitive set	Public approval and sales data	12 approved therapies; 3 US approvals in 2025; ~70% of US patients prefer oral prophylaxis. Counterpoint for the	Yes

Claim	How checked	Found	Confirmed?
		record: HAELO's 62% attack-free <i>and</i> therapy-free is a direct answer to the oral-preference argument, because an oral pill is still daily chronic therapy.	
Commercial track record	FDA product databases	Zero approvals, zero recalls, zero enforcement — because nothing has ever shipped. Clean here is not clean; it is an absence of evidence about a company about to run its own launch.	Yes
Patent/exclusivity risk	Correct database	lonvo-z is a biologic, so the Orange Book does not apply; the Purple Book is the venue and there is no listing pre-approval. No near-term generic exposure; 12 years of exclusivity if approved.	Yes
Fixed cost base	Property disclosure	~502,000 sq ft leased across Cambridge/Waltham. Consistent internally, but a large fixed obligation for a company with no revenue.	Yes
Short interest at ~38%	Secondary source only	Not verified against primary data. The FINRA read was not run. Treat the 38% as approximately right, not established.	No — open item
Live price	Broker connection	Permission not granted this session. No live tick. Price corroborated at \$12.84, the 2026-08-21 close, matching the evidence pack.	No — flagged, not skipped
Next earnings date	Checked for a company announcement	2026-11-05 is derived from a data vendor. No Intellia release schedules it , including the 8/6 Q2 release.	No — unconfirmed
Description of the therapy	Language check	One bench described the treatment as “germline-adjacent.” It is not: this is a one-time edit of liver cells in a living adult, not heritable. Sloppy phrasing that overstates the risk narrative.	Refuted — our own bench's wording error

PROPOSED ENTRY BANDS

No purchase today at \$12.84. The tape is above our accumulation band and only 3% below the price at which the company itself has been selling stock.

Action	Price / condition	Size	Reasoning
Do nothing now	\$12.84 (2026-08-21 close)	0%	Above the band. Buying here means buying into \$368.5M of open issuer supply, 3% under the

Action	Price / condition	Size	Reasoning
			company's own average sale price.
First tranche	\$11.75 or lower	0.25–0.375% of the book	Top of the zone between the company's April offering price and its at-the-market average. Below this, we are paying less than the last institutional clearing prices.
Second tranche	\$10.75 or lower, or on FDA acceptance of the lonvo-z filing (whichever comes first)	additional 0.25–0.375%	\$10.75 is management's own clearing price with a book open, post-data. Below it, the at-the-market program is effectively out of the money and the valuation table pays.
Total starter size	—	0.50–0.75% of the book, maximum	Small deliberately: our value estimate is not meaningfully different from the Street's, and the outcome is binary on commercial adoption.
Hard no-buy line	\$13.21	—	The average price the company itself sold at. We do not pay more than the issuer accepts.
Trim	\$18.80+	half	Top of the defensible valuation range even on the generous 4× sales multiple. Above this we are paying for ATTR before the expanded trial reads.
Trim	\$25.90+	balance	Requires \$1.2B peak HAE sales — roughly 2.4× the Street's bear anchor. Sell into that, don't model it.
Kill (exit fully, no re-entry without new work)	FDA refuses to file the lonvo-z application	—	Removes the near-term funded asset.
Kill	Any Grade 3 or worse liver enzyme event at the	—	Converts the safety issue from managed to unresolved.

Action	Price / condition	Size	Reasoning
	resumed dose in the expanded ATTR trial		
Kill / re-underwrite	Any at-the-market sale disclosed below \$11.00	—	Signals the company is funding into weakness, and puts supply directly on top of our band.

Catalyst dates:

Date	Event	Confirmed?
Second half of 2026 (no specific date)	FDA acceptance of the lonvo-z application, announced by 8-K. This is the binding catalyst and it is undated.	Company guidance; no date
Second half of 2026 (no specific date)	Expanded ATTR (MAGNITUDE) enrollment completion	Company guidance; no date
2026-11-05	Q3 earnings	Unconfirmed — data-vendor derived; no company release schedules it
First half of 2027	Planned lonvo-z commercial launch	Company guidance

Why staged rather than all-at-once: the binding catalyst has no date, the issuer sells into strength, and 38% of the float is short. That combination produces violent moves in both directions. Tranches are the only defense against being early on an undated event.

ARMED SENSORS

(armed provisionally with this deck; session adjudication ratifies)

- price alert: fires below 11.0 (deck-provisional: Re-underwrite trigger if an at-the-market sale prints here — issuer funding into weakness on top of ou)
- filing watch: 8-K until 2027-02-18 (deck-provisional: FDA acceptance or refusal-to-file of the lonvo-z BLA — the undated binding catalyst, guided 2H26)
- dated pack NTLA|2026-12-31: NTLA: End of the guided 2H26 window for FDA acceptance of the lonvo-z BLA — no specific date exists; this is a window boundary, not an event
- dated pack NTLA|2026-11-05: NTLA: Q3 2026 earnings — data-vendor derived, no Intellia release schedules it (date confirmed=False)
- dated pack NTLA|2027-06-30: NTLA: End of guided 1H27 window for lonvo-z commercial launch, Intellia's first ever product launch (date confirmed=False)

What remains unverified

Stated plainly, so nobody treats this deck as more solid than it is.

1. **The valuation multiple, which is the entire result.** We used 2.5× peak annual sales for the HAE franchise, on the reasoning that a one-time cure earns a bolus from the existing patient population and then a thin trickle from new diagnoses. That number has **no source**. At 3.5× the answer is \$16.98; at 4×, \$18.80. Nothing in this deck resolves which is right, and the difference is the whole investment case. This is the top of the work queue.
 2. **The value of the ATTR program.** We assigned it zero. That is deliberately conservative and almost certainly wrong in the company's favor — it is a Phase 3 asset, 75% owned, in a market where one branded drug does over \$5 billion a year. But we have no probability of success, no timeline (the trial just grew 57%), and no pricing assumption. “Free option” is a phrase, not a number.
 3. **The FDA acceptance date.** Guided to the second half of 2026. No date. Our second tranche is partly gated on an event we cannot schedule.
 4. **The Q3 earnings date.** 2026-11-05 comes from a data vendor. No company release confirms it.
 5. **Short interest.** ~38% from a secondary source. The primary regulatory read was not run.
 6. **The live price.** Broker access was denied this session. Everything here uses the 2026-08-21 close of \$12.84. Before any order, re-price.
 7. **Peak sales for lonvo-z.** We leaned on Evercore ISI's \$500M as the bear anchor. We did not independently model HAE prevalence, treatment-eligible population, pricing, or the share a one-time therapy takes from twelve entrenched competitors against a documented oral preference. This is the number that decides everything and we have not done our own work on it.
 8. **Pricing and reimbursement.** No verified datapoint on what a one-time HAE cure will be paid, or whether payers will fund it up front for a non-fatal, well-managed disease. A credible figure above \$1.0M per patient with an outcomes-based contract would move our estimate materially.
 9. **The liver safety mechanism.** The FDA-aligned mitigation package includes a real prospective exclusion for patients with certain liver abnormalities. But the underlying susceptibility — including the retrospective genetic correlation — is not understood, and no prospective genetic screen exists. We cannot say the risk is characterized; we can say it is being managed.
 10. **Launch execution.** Intellia has never sold a product. Nothing in this analysis models the probability that a first commercial launch underperforms, and general and administrative spend rising 39% year-over-year tells us the build is happening, not that it will work.
 11. **Whether we should own this at all.** Two of our own reviewers reached “small starter” from opposite directions — one from “fair value, own the minimum,” one from “the bear case is overstated, own a little on a pullback.” Neither reached “buy it here.” That agreement on *not now* is the most robust output of this exercise.
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